

PATIENT

Mr. Ramesh Chandra Gupta

68 Years | Male | Retired Civil Engineer

Blood Group: A+ | Weight: 74 kg | Height: 168 cm

Known Conditions: HTN (12 yrs), T2DM (8 yrs)

UHID

FH-GGN-2024-74821

Referral Doctor

Dr. Atul Kumar, DM Nephrology

Ward / OPD

Nephrology OPD – Room 412

Sample Collected

05 Jun 2026 06:45 AM

Report Generated

05 Jun 2026 12:00 PM

Clinical Priority

URGENT

CKD STAGE ASSESSMENT

eGFR: 22 mL/min/1.73m² → CKD Stage 4 (Severely Decreased)

URGENT REVIEW

RENAL FUNCTION TESTS

Panel: RFT-001 | URGENT

INVESTIGATION	RESULT	UNIT	REFERENCE RANGE	STATUS
Serum Creatinine	4.82	mg/dL	0.7 – 1.2	CRITICAL ▲
Blood Urea Nitrogen (BUN)	88	mg/dL	7 – 25	CRITICAL ▲
Blood Urea	188	mg/dL	15 – 55	CRITICAL ▲
eGFR (CKD-EPI 2021)	22	mL/min/1.73m ²	≥ 60	CRITICAL ▲
Uric Acid	9.2	mg/dL	3.5 – 7.2	HIGH ▲
Cystatin C	3.1	mg/L	0.5 – 1.0	CRITICAL ▲
Beta-2 Microglobulin	8.4	mg/L	0.8 – 2.4	CRITICAL ▲
Urine Protein / Creatinine Ratio	3.8	g/g	< 0.2	CRITICAL ▲
Urine Albumin (24-hr)	2850	mg/day	< 30	CRITICAL ▲
Urine Output (24-hr)	920	mL/day	800–2000	LOW ▼

ELECTROLYTES & ACID-BASE

Panel: ELEC-002

INVESTIGATION	RESULT	UNIT	REFERENCE RANGE	STATUS
Serum Sodium (Na+)	132	mEq/L	136 – 145	LOW ▼
Serum Potassium (K+)	6.1	mEq/L	3.5 – 5.1	CRITICAL ▲
Serum Chloride (Cl-)	95	mEq/L	98 – 107	LOW ▼
Serum Bicarbonate (HCO ₃ -)	16	mEq/L	22 – 29	LOW ▼
Serum Phosphorus	6.8	mg/dL	2.5 – 4.5	HIGH ▲
Serum Calcium (Total)	7.4	mg/dL	8.5 – 10.5	LOW ▼
Ionized Calcium	3.6	mg/dL	4.5 – 5.3	LOW ▼
Serum Magnesium	3.1	mg/dL	1.7 – 2.2	HIGH ▲

Anion Gap	21	mEq/L	8 – 16	HIGH ▲
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HAEMATOLOGY (Anaemia of CKD)				Panel: HEM-003
INVESTIGATION	RESULT	UNIT	REFERENCE RANGE	STATUS
Haemoglobin	7.8	g/dL	13.0 – 17.0	CRITICAL ▲
Haematocrit	23	%	40.0 – 54.0	CRITICAL ▲
RBC Count	2.8	x10 ¹² /uL	4.5 – 5.9	LOW ▼
Serum Ferritin	18	ng/mL	30 – 400	LOW ▼
Serum Iron	42	ug/dL	60 – 170	LOW ▼
TIBC	380	ug/dL	250 – 370	HIGH ▲
Transferrin Saturation	11	%	20 – 50	LOW ▼
EPO (Erythropoietin)	6.2	mIU/mL	4.3 – 29.0	Normal
PTH (Intact)	485	pg/mL	15 – 65	CRITICAL ▲
Vitamin D (25-OH)	8	ng/mL	30 – 100	LOW ▼

NEPHROLOGIST'S CLINICAL IMPRESSION
DIAGNOSIS: Chronic Kidney Disease – Stage 4 (eGFR 22) Hypertensive + Diabetic Nephropathy 1. CKD Stage 4 – Imminent Renal Replacement Therapy (RRT) — Creatinine 4.82, BUN 88, eGFR 22. Patient approaching ESRD. Nephrology review and AV fistula creation planning to be initiated. 2. Hyperkalaemia – LIFE THREATENING (K+ 6.1 mEq/L) — Requires immediate IV calcium gluconate, salbutamol nebulisation, and potassium-restricted diet. ECG to rule out cardiac toxicity. 3. Metabolic Acidosis — HCO3 16, Anion Gap 21. Oral sodium bicarbonate supplementation (0.5–1 mEq/kg/day) indicated to slow CKD progression. 4. Severe Renal Anaemia — Hb 7.8 g/dL with iron deficiency (Ferritin 18, TSAT 11%). IV iron sucrose + erythropoiesis-stimulating agent (ESA) recommended. Target Hb 10–11.5 g/dL. 5. Secondary Hyperparathyroidism — PTH 485 pg/mL with severe Vit D deficiency. Calcimimetics (Cinacalcet) + active Vit D (Calcitriol 0.25 mcg/day). Low phosphorus diet.

Dr. Atul Kumar, MD DM Sr. Consultant – Nephrology Fortis Gurugram MCI: 61209	Dr. Vandana Mehta, MD Head – Clinical Chemistry Fortis Lab Services MCI: 48802	05 Jun 2026 12:00 PM Digital Auth: Fortis LIS 5.1 Report Status: FINAL Priority: URGENT
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